

Name : Mr. PRAVEEN

Age/Gender : 30 Years / Male

Ref.By : Self

Req No. : BNG2657096

Sample Type : Serum

Client Name : TEZHEALTH PRIVATE LIMITED-CMLKAD56

Vial ID : 3139055

Collected On : 28-May-2026 02:58 PM

Registered On : 28-May-2026 02:58 PM

Reported On : 28-May-2026 05:09 PM

Client Code : CMLKAD56

Kidney Basic Screen

Test Name	Observed Values	Units	Biological Reference Intervals
* SERUM CREATININE Method:Enzymatic Method (sarcosine oxidase, Peroxidase)	1.08	mg/dL	0.6 - 1.4
* UREA Method:Urease UV	28.49	mg/dL	16.8-43.2
* UREA NITROGEN (BUN) Method:Calculated	13.31	mg/dL	8 - 23
* BUN/CREATININE RATIO	12.33		
* URIC ACID Method:Uricase-Peroxidase	5.63	mg/dL	Male:3.6 -8.2 Female:2.3 - 6.1
* SODIUM Method:ISE Direct	140	mmol/L	136-145
* POTASSIUM Method:ISE Direct	3.89	mmol/L	3.5 - 5.5
* CHLORIDE Method:ISE Direct	101	mmol/L	95-105

INTERPRETATION::

kidney disease usually does not have signs or symptoms. Testing is the only way to know how your kidneys are doing. It is important to get checked for kidney disease if an individual having the key risk factors - diabetes, high blood pressure, heart disease, or a family history of kidney failure.



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KMC.No:103490
MD PATHOLOGY

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LIVER FUNCTION TEST (LFT)

Test Name	Observed Values	Units	Biological Reference Intervals
* TOTAL BILIRUBIN Method:Diazonium Ion	0.70	mg/dL	0.3 - 1.2
* DIRECT BILIRUBIN Method:Diazonium	0.18	mg/dL	0.0 - 0.2
* INDIRECT BILIRUBIN Method:Calculated	0.52	mg/dL	0.2 - 0.8
* SGOT / AST Method:IFCC without pyridoxaal phosphate	32.7	IU/L	Upto 35
* SGPT / ALT Method:Tris buffer without p5p	47.8	IU/L	Upto 45
* Alkaline Phosphatase(ALP) Method:AMP BUFFER METHOD	86.7	U/L	30-120
* PROTEIN-TOTAL Method:Biurate	7.65	g/dL	6.00 - 8.00
* ALBUMIN Method:BCG	4.33	g/dL	4.0-5.5
* GLOBULIN Method:Calculated	3.32	g/dL	2.5 - 3.5
* A/G RATIO Method:Calculation	1.30	-	1.0 - 2.1
* GAMMA GLUTAMYL TRANSFERASE Method:IFCC stand	28.2	U/L	Male :11-61 Female :9-39



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Lipid Profile

Test Name	Observed Values	Units	Biological Reference Intervals
* TOTAL CHOLESTEROL Method:CHOD-POD Method	200.9	mg/dL	Desirable : < 200 Borderline: 200 - 239 High Risk : > 240
* HDL - CHOLESTEROL Method:Direct Method	47.2	mg/dL	Desirable Level :>59 Optimal :40-59 Undesirable :<40
* TRIGLYCERIDES Method:GPO-POD Method	173.8	mg/dL	<203.5
* VLDL CHOLESTEROL	34.76	mg/dL	< 30
* LDL - CHOLESTEROL	118.94	mg/dL	Optimal :<100 near Optimal :100-129 Borderline High:130-159 High :160-189 Very High :>189
* CHOL / HDL Ratio.	4.26	-	-
* HDL/LDL CHOLESTEROL RATIO	0.40	Ratio	-
* LDL / HDL RATIO	2.52	Ratio	Desirable level :0.5 3.0 Borderline Risk :3.0-6.0 High Risk :>6.0



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Iron Deficiency Profile

Test Name	Observed Values	Units	Biological Reference Intervals
* IRON Method: Colorimetric Assay	83	µg/dL	Male: 45-158 Female: 37-145
* Total Iron Binding Capacity (TIBC) Method: Ferrozine	334.2	µg/dL	240 - 450
* TRANSFERRIN Method: Immunoturbidometry	227.35	ug/dl	176 - 280
* TRANSFERRIN SATURATION	24.84	%	20 - 50
* FERRITIN Method: Chemiluminescence	96.2	ng/mL	11.0 - 306.8

INTERPRETATION::

Iron is a mineral that's essential for making red blood cells. Red blood cells carry oxygen from the lungs to the rest of the body. Iron is also important for healthy muscles, bone marrow, and organ function. Iron levels that are too low or too high can cause serious health problems. Iron levels that are too low include Pale skin, Fatigue, Weakness, Dizziness, Shortness of breath, Rapid heartbeat. Iron levels that are too high include Joint pain, Abdominal pain, Lack of energy, Weight loss.



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THYROID PANEL-I

Test Name	Observed Values	Units	Biological Reference Intervals
* TOTAL TRIIODOTHYRONINE (T3) Method:CLIA	0.93	ng/mL	0.58 - 1.62
* TOTAL THYROXINE (T4) Method:CLIA	7.44	ug/dl	5.0 - 14.5
* Thyroid Stimulating Hormone.(TSH) Method:CLIA	6.29	μIU/mL	0.35 – 5.1

INTREPRATATION:

The determination of T3 is utilized in the diagnosis of T3hyperthyroidism,the detection of early stages of hyperthyroidism and for indicating a diagnosis of thyrotoxicosis factitia.

The determination of T4 can be utilized for the following indications: the detection of hyperthyroidism, the detection of primary and secondary hypothyroidism, and the monitoring of TSH-suppression therapy.

The determination of TSH serves as the initial test in thyroid diagnostics.Even very slight changes in the concentrations of the free thyroid hormones bring about much greater opposite changes in the TSH level. Accordingly,TSH is a very sensitive and specific parameter for assessing thyroid function and is particularly suitable for early detection or exclusion of disorders in the central regulating circuit between the hypothalamus, pituitary and thyroid.



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Sample Type : Plasma-R

Client Name : TEZHEALTH PRIVATE LIMITED-CMLKAD56

Vial ID : 3139053

Collected On : 28-May-2026 02:58 PM

Registered On : 28-May-2026 02:58 PM

Reported On : 28-May-2026 05:08 PM

Client Code : CMLKAD56

Random Blood Sugar-Glucose (RBS)

Test Name	Observed Values	Units	Biological Reference Intervals
* RANDOM BLOOD GLUCOSE	75.3	mg/dL	70 - 140

Method : Spectrophotometry**INTERPRETATION::**

Symptoms of high blood glucose levels include Increased thirst, more frequent urination, Blurred vision, Fatigue, Wounds that are slow to heal

Symptoms of low blood glucose levels include Anxiety, Sweating, Trembling, Hunger, Confusion.



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25-Hydroxy Vitamin D

Test Name	Observed Values	Units	Biological Reference Intervals
* 25-OH VITAMIN (VIT D3) Method:C L I A	38.78	ng/mL	Deficiency: <10 Insufficiency: 10-30 Sufficiency:30-100 Toxicity: >100

INTERPRETATION::

Vitamin D is a nutrient that is essential for healthy bones and teeth. There are two forms of vitamin D that are important for nutrition: vitamin D2 and vitamin D3. Vitamin D2 mainly comes from fortified foods like breakfast cereals, milk, and other dairy items. Vitamin D3 is made by your own body when you are exposed to sunlight. It is also found in some foods, including eggs and fatty fish, such as salmon, tuna, and mackerel.

Vitamin D deficiency is due to:

- Not getting enough exposure to sunlight
- Not getting enough vitamin D in your diet
- Having trouble absorbing vitamin D in your food

A low result may also observed due to body is having trouble using the vitamin as it should, and may indicate kidney or liver disease.

A vitamin D deficiency is usually treated with supplements and/or dietary changes.



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Sample Type : WB-EDTA

Client Name : TEZHEALTH PRIVATE LIMITED-CMLKAD56

Vial ID : 3139054

Collected On : 28-May-2026 02:58 PM

Registered On : 28-May-2026 02:58 PM

Reported On : 28-May-2026 03:12 PM

Client Code : CMLKAD56

Complete Blood Count (CBC)

Test Name	Observed Values	Units	Biological Reference Intervals
* HAEMOGLOBIN Method:Colorimetric	15.1	gm%	14.0 -17.0
* RBC Count Method:Electrical Impedence	5.30	millions/cumm	4.50 - 5.50
* WBC Count Method:Electrical Impedence	8400	cells/cumm	4000-11000
* PLATELET COUNT Method:Impedence and Light Scattering	2.43	lakhs/cumm	1.5 - 4.5
* Packed Cell Volume(PCV) Method:Cum.RBC Pulse High detection	43.9	%	40 - 50
* MCV Method:Calculated	83	fL	80 - 100
* MCH Method:Calculated	28.5	pg	27 - 33
* MCHC Method:Calculated	34.0	g/dL	32.0 - 34.0

DIFFERENTIAL COUNT

* NEUTROPHILS,, Method:Manual - Leishman Stain & Smear Microscopy	51	%	50-75
* LYMPHOCYTES Method:Flowcytometry	40	%	20 - 40
* EOSINOPHILS Method:Manual - Leishman Stain & Smear Microscopy	04	%	02 - 06
* MONOCYTES Method:Flowcytometry	05	%	02 - 08
* BASOPHILS Method:Manual - Leishman Stain & Smear Microscopy	00	%	0 - 1



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Vitamin - B12

Test Name	Observed Values	Units	Biological Reference Intervals
* Vitamin - B12	433.9	pg/mL	180 - 916

Method:CLIA

INTREPRATATION:

Vitamin B12 and folate are critical to normal DNA synthesis, which in turn affects erythrocyte maturation.3 Vitamin B12 is also necessary for myelin sheath formation and maintenance. The body uses its B12 stores very economically, reabsorbing vitamin B12 from the ileum and returning it to the liver so that very little is excreted. Clinical and laboratory findings for B12 deficiency include neurological abnormalities, decreased serum B12 levels, and increased excretion of methylmalonic acid. The impaired DNA synthesis associated with vitamin B12 deficiency causes macrocytic anemias. These anemias are characterized by abnormal maturation of erythrocyte precursors in the bone marrow, which results in the presence of megaloblasts and in decreased erythrocyte survival. Pernicious anemia is a macrocytic anemia caused by vitamin B12 deficiency that is due to lack of intrinsic factor. Low vitamin B12 intake, gastrectomy, diseases of the small intestine, malabsorption, and trans-cobalamin deficiency can also cause vitamin B12 deficiency.



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Glycosylated Hemoglobin (GHb/HbA1c)

Test Name	Observed Values	Units	Biological Reference Intervals
* GLYCOSYLATED HAEMOGLOBIN (HbA1C) Method:Enzymatic method	5.95	%	Non-diabetic: 4.8 - 5.9 % Pre-diabetic: 5.9 - 6.5 % Diabetic: >= 6.5 %
* Approximate mean plasma glucose Method:Calculated	124.07		<111

INTERPRETATION::

A hemoglobin A1c (HbA1c) test measures the amount of blood sugar (glucose) attached to hemoglobin. If HbA1c levels are high, it may be a sign of diabetes, a chronic condition that can cause serious health problems, including heart disease, kidney disease, and nerve damage. Note: The HbA1c test is not used for gestational diabetes, a type of diabetes that only affects pregnant women, or for diagnosing diabetes in children. References American Diabetes Association. Standards of medical care in diabetes—2014. Diabetes Care. 2014 Jan;37 Suppl 1:S14-80.	HbA1C Value in %	Average Glucose in mg/dL	Impression
	4.0	68	Non-Diabetic
	4.5	82	
	5.0	97	
	5.5	111	
	6.0	125	Pre-Diabetic/Good Control
	6.5	140	
	7.0	154	Diabetic/Poor Control
	7.5	169	
	8.0	183	
	8.5	197	
	9.0	212	
	9.5	226	
	10.0	240	



Signature

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Calcium

Test Name	Observed Values	Units	Biological Reference Intervals
* CALCIUM	9.48	mg/dL	8.8-10.62

Method: Arsenazo III Method

INTERPRETATION::

Calcium is one of the most important minerals in the body. Calcium needed for healthy bones and teeth. Calcium is also essential for proper functioning of the nerves, muscles, and heart.

Symptoms of high calcium levels include:

- Nausea and vomiting
- More frequent urination
- Increased thirst

• Constipation

• Abdominal pain

• Loss of appetite

Symptoms of low calcium levels include:

• Tingling in the lips, tongue, fingers, and feet

• Muscle cramps

• Muscle spasms

• Irregular heartbeat



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